FOR WOMEN 30+ WHO WERE TOLD "EVERYTHING LOOKS FINE"

Normal Isn't Fine

The 15-minute plan to get heard when your labs say "normal" and you still feel off. Word-for-word scripts included.

The Kira Method · 3-Fact Opener · Symptom log · One-page brief · Scripts · After-visit sheet

READ THIS FIRST

Normal isn't the same as fine

You have 15 minutes with your doctor. You have been tired, bloated, foggy or awake at 3 a.m. for months. You say it, the labs come back "normal," and you leave with no answers.

That is a common experience. In a 2026 survey by the Flo Health app, 53 percent of the 1,493 women who described themselves as perimenopausal said their symptoms had been dismissed or not taken seriously. In a 2025 survey commissioned by the company Biote, 39 percent of the women asked felt they had been misdiagnosed. Both surveys come from companies that sell to women, and the people surveyed chose to answer, so treat the numbers as a signal, not as a statistic for all women. The signal is clear enough: many women walk out of appointments feeling unheard.

> **What this kit does.** It does not diagnose you and it does not tell your doctor what to do. It helps you describe what is happening clearly, in the time you have, so that your doctor can do their best work. Clear information leads to better questions, and better questions lead to better care.

> **This is education, not medical advice.** If you have chest pain, trouble breathing, fainting, sudden weakness on one side, confusion, severe pain, heavy bleeding that soaks through a pad or tampon every hour, or blood in your stool or urine, do not wait for an appointment. Call your doctor now or go to emergency care.

How to use it (about 20 minutes of prep, plus 2 weeks of notes)

1. Start the **14-day symptom log** (Part 2) two weeks before your visit, if you can.
2. Fill in your **one-page Doctor Brief** (Part 3) the night before.
3. Pick your **top three concerns** and practice the **scripts** (Part 4) out loud once.
4. Use the **question bank** (Part 5) and the **"labs are normal" guide** (Part 6).
5. After the visit, fill in the **After-Visit Sheet** (Part 8) in the parking lot or the car, while it is fresh.

PART 1

The 10-minute prep that changes the visit

Two U.S. government health agencies, the Agency for Healthcare Research and Quality (AHRQ) and the National Institute on Aging (NIA), give the same core advice, and it works:

- **Pick your top three.** Choose the three or four concerns you most want to talk about, and say them first. If you wait until the end, the doctor may already have a hand on the door.
- **Describe each symptom in three facts:** when it started, how often it happens, and whether it is getting better or worse.
- **Bring every medicine and supplement** you take, including vitamins, herbal products and over-the-counter medicines. Bring the bottles or a photo of the labels.
- **Write your questions down.** Nerves make people forget. A list in your hand also shows you are serious.
- **Bring someone** if you can, to take notes. If not, ask if you may record the key points on your phone, and write them down at once.

The 3-Fact Opener

> **"For [how long], I have had [symptom]. It happens [how often]. It is [getting worse / staying the same / getting better]. It affects my [sleep / work / mood / daily life] by [one real example]."**

Example: "For four months, I have been waking up at 3 a.m. almost every night and cannot get back to sleep. It is getting worse. I am so tired at work that I rely on four cups of coffee, and I snapped at my daughter last week."

Why the real example matters. "I am tired" is hard to act on. "I fall asleep at my desk at 2 p.m. and I had to stop driving at night" tells your doctor how serious it is.

PART 2

14-day symptom log

Fill this in at the same time each evening, in two minutes. Patterns are your best evidence. Rate each item from 0 (none) to 5 (severe). Circle the day your period starts, if you still have one.

Day	Sleep (hours)	Woke at night?	Energy 0–5	Bloating 0–5	Mood 0–5	Hot flush / sweats	Notes
1							
2							
3							
4							
5							
6							
7							

Day	Sleep (hours)	Woke at night?	Energy 0–5	Bloating 0–5	Mood 0–5	Hot flush / sweats	Notes
8							
9							
10							
11							
12							
13							
14							

Also note: caffeine and alcohol, big meals late at night, stress, medicines, and your period (dates, how heavy, how regular). Write the pattern you see on your Doctor Brief.

PART 3

Your one-page Doctor Brief

Fill it in, print it, and hand a copy to your doctor at the start. Many doctors appreciate a single clear page.

Name / date of birth	
My top 3 concerns (most important first)	1. 2. 3.
What is happening (3 facts each: when it started, how often, better or worse)	
How it affects my life (one real example)	
Pattern I noticed in my log	
My periods (regular? last period? changes?)	
Medicines, vitamins and supplements (name and dose)	
What I have already tried, and what happened	
Family history I know of (heart, thyroid, diabetes, cancer, early menopause)	
My goal for today	(example: "a clear next step, and to know what has been ruled out")

PART 4

What to say: word-for-word scripts

Say it calmly. You are not arguing, you are building a plan together.

Starting the visit

"Thank you for seeing me. I have three things I would like to cover, and I wrote them down so I do not forget. The most important one is [concern 1]. May I give you a one-page summary?"

When you hear "your labs are normal"

"I am glad those are normal. But I still have [symptom], and it is affecting [real example]. What else could be causing it, and what do you suggest we do next?"

When you feel brushed off

"I hear you. I would feel better with a clear plan. What would you want to rule out first, and when should I come back if nothing changes?"

When you want a test and the answer is no

"Can you help me understand why that test is not helpful for me? And could you note in my chart that we discussed it?"

When you want to know what is ruled out

"What are the most likely causes, what have we ruled out so far, and what is still possible?"

When you want a specialist

"Would it make sense to see a [gynecologist / endocrinologist / sleep specialist / dietitian] for this? Could you refer me, or tell me what would make a referral appropriate?"

Closing the visit

"Let me repeat the plan to be sure I have it right: [plan]. When will I hear about results, and what should I do if I get worse before then?"

If you feel unheard even after this, it is reasonable to ask for a second opinion or to see a different provider. That is your right.

PART 5

Question bank

AHRQ suggests starting with a few questions you can use at almost any visit. Tick the ones you want to ask, and write down the answers.

For any test, medicine or plan

☐ What is this test for, and what will the result change?

☐ When will I get the results, and how will I hear?

☐ What are the side effects, and does it interact with anything I already take?

☐ What happens if I wait and do nothing?

☐ Are there other options?

If you are always tired

☐ Which common causes have we checked, and which have we not? (For example blood count, thyroid, blood sugar, iron, B12, vitamin D.)

☐ Could my sleep be the problem? Is a sleep assessment worth doing?

☐ Could any medicine I take be adding to this?

☐ Could mood or stress be part of this, and can we screen for it?

If you wake at 3 a.m. or sleep badly

☐ Could this be related to perimenopause, stress, alcohol, caffeine, or sleep apnea?

☐ What should I try first, and for how long before we reassess?

If you are bloated or have belly changes

☐ What could cause this, and which symptoms would make you want to investigate sooner?

☐ Is a change in my eating, fiber or medicines worth trying first?

If your periods are changing

☐ Could this be perimenopause, and what else should we rule out?

☐ What changes in my bleeding should make me call you quickly?

If your weight or energy changed fast

☐ What could explain this, and should we check anything before I change my plan?

PART 6

"Your labs are normal." Now what?

Normal results are good news. They mean certain problems are less likely. They do not mean you imagined your symptoms. A lab "normal range" is built from many people, and many things that make you feel unwell do not show up on a basic panel.

Questions to ask about the lab work

- Which tests were done? Can I have a copy of the results?
- Common first checks for ongoing tiredness include a blood count, thyroid function, blood sugar, iron, vitamin B12 and vitamin D. **Ask which of these were checked, and which are right for you.** Your doctor decides, and not every woman needs every test.
- If everything is normal, what are the next steps? Sleep, mood, medicines, and lifestyle are often the next place to look.

If you think it may be perimenopause

Perimenopause often starts in the 40s and sometimes the mid-30s. Common signs include irregular periods, hot flashes, night sweats, sleep trouble, mood changes and brain fog. UK national guidance (NICE) says that in otherwise healthy women over 45, perimenopause can be diagnosed from the story of your symptoms, and that hormone blood tests such as FSH do not help with the diagnosis because hormone levels go up and down. The Cleveland Clinic says the same: one test can look "normal" one week and different the next. Hormone testing becomes more useful in specific cases, for example when periods stop before age 45.

> **So the most useful thing you can bring is your log.** Patterns of sleep, hot flushes, mood and period changes over several weeks are the real evidence.

> **A note about at-home hormone tests.** Many are sold online, and some of the sellers also sell treatment. Because hormone levels fluctuate in perimenopause, a single home result may be misleading. Talk to a clinician before you rely on one or make decisions from it.

Look-alikes. Thyroid problems, low iron, sleep apnea, depression and anxiety can cause symptoms similar to perimenopause. That is why your doctor may ask many questions. **Do not wait** if you have chest pain, shortness of breath, fainting, confusion, weakness on one side, blood in the stool or urine, or unexplained weight loss: seek care promptly.

PART 7

Second opinions, referrals and better care

- **Primary care provider:** the starting point for most symptoms, and the person who can refer you.
- **Gynecologist or a menopause specialist:** if you suspect perimenopause or menopause. The Menopause Society (formerly NAMS) lists clinicians who hold a Menopause Society Certified Practitioner credential. Check their website for a directory.
- **Endocrinologist:** for thyroid, diabetes or hormone questions.
- **Sleep specialist:** if snoring, gasping, or long-lasting tiredness despite enough hours in bed.
- **Registered dietitian:** for food, weight, protein or blood sugar questions.

> **How to ask for a second opinion kindly:** "I would like to get a second opinion so I feel confident in the plan. Could you send my records to [clinic], or tell me how to request them?"

Practical tips

- Book the **first or last slot** of the morning or afternoon if you can: fewer delays, more time.
- Say at booking: "I would like a longer visit to discuss ongoing symptoms."
- Use the **patient portal** to read results and send short follow-up questions.
- For telehealth visits, have your log and brief open on screen, and write down the plan.
- You can ask for a copy of your records. In the U.S. you generally have the right to access them.

PART 8

After-Visit Sheet

Fill it in right after the visit, or within an hour.

Date / provider	
What the doctor thinks is going on	
What has been ruled out	
Tests ordered (what, where, when)	
Medicines or changes to start or stop	
Referrals	
When results will come, and how	
Follow-up date	
Call or return sooner if...	
Things I did not get answered	

My next step this week

SOURCES

Where this comes from

Written October 2026. Guidelines change, so check with your own care team.

- Agency for Healthcare Research and Quality (AHRQ), "Questions Are the Answer" and "The 10 Questions You Should Know" (ahrq.gov).
- National Institute on Aging (NIA), "Tips for Talking With Your Doctor" (nia.nih.gov).
- NICE (UK), Menopause quality standard QS143: diagnosing perimenopause and menopause from clinical history in women over 45; hormone tests such as FSH not recommended for diagnosis.
- Cleveland Clinic, "Perimenopause" overview (my.clevelandclinic.org).
- Flo Health survey (September 2026): 1,493 U.S. app users who described themselves as perimenopausal. Self-selected sample, company survey.
- Biote "Perimenopause Focus" survey (2025): more than 1,000 U.S. working women aged 30–60. Commissioned by a company that sells hormone-related products.

> This kit is educational. It is not medical advice, diagnosis or treatment, and it does not create a doctor–patient relationship. Always follow your own doctor's guidance. Individual results vary and no specific outcome is promised.

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